

Wisconsin Department of Health Services

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 0018302	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED C 08/25/2026
NAME OF PROVIDER OR SUPPLIER WELLINGTON PLACE AT RIB MOUNTAIN		STREET ADDRESS, CITY, STATE, ZIP CODE 149500 COUNTY ROAD NN WAUSAU, WI 54401		
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N 000	Initial Comments On 08/13/2026, Surveyor conducted 2 complaint investigations at Wellington Place at Rib Mountain. Data was collected through 08/25/2026. One of the 2 complaints was substantiated. Seven deficiencies were identified. One of the seven deficiencies was a repeat deficiency. See Statement of Deficiency JBS011, dated 01/29/2024. Census: 21	N 000		
N 214	83.15(3)(a) Administrator shall supervise daily operation The administrator shall supervise the daily operation of the CBRF, including but not limited to, resident care and services, personnel, finances, and physical plant. The administrator shall provide the supervision necessary to ensure that the residents receive proper care and treatment, that their health and safety are protected and promoted and that their rights are respected. This Rule is not met as evidenced by: Based on record review and interviews, the provider did not ensure an administrator supervised the daily operations of the Community Based Residential Facility (CBRF), to ensure residents received proper care and treatment, that their health and safety were protected and	N 214		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

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N 214	Continued From page 1 promoted, and their rights were respected. Findings include: This provider is licensed to care for up to 30 residents in the client groups of: irreversible dementia/Alzheimer's disease, advanced age, and physically disabled. On 07/30/2026, the Department received a complaint with allegations the provider did not ensure a resident received the cares and services s/he required and s/he developed multiple wounds. From 07/08/2026 until 07/14/2026, the Department conducted a survey and identified the provider had not notified the Department of an administrator change; see Statement of Deficiency (SOD) WUG114, dated 07/14/2026. During this survey, the provider indicated Administrator D was no longer involved with the program and that Administrator A oversaw the daily operations. Administrator D indicated s/he was available as a resource to Administrator A. On 08/10/2026, Surveyor reviewed the Department's records for the provider. Administrator D was still listed as the administrator of the facility. On 08/13/2026 at approximately 10:00 a.m., Surveyor interviewed Care Coordinator (CC) E. CC E told Surveyor Administrator A was the administrator for the facility. From 08/13/2026 until 08/25/2026, Surveyor had multiple email correspondence between Administrator A and Chief Operating Officer (COO) B. Administrator A's email correspondences signature indicated his/her title as administrator. COO B's email signature indicated s/he resided in Colorado.	N 214		

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N 214	Continued From page 2 Throughout the course of the complaint investigation, the department found the provider did not: - Maintain documentation of Administrator A's training. - Complete a thorough pre-admission assessment to determine if the provider could meet Resident 1's needs. - Create a temporary service plan to meet Resident 1's immediate needs. - Update Resident 1's individual service plan (ISP) upon changes in condition. - Maintain accurate medication administration records. - Provide adequate and appropriate care to Resident 1. From 08/13/2026 until 08/25/2026, Surveyor reviewed documentation from Resident 1's record from his/her previous stay at a skilled nursing facility, his/her stay at Wellington Place at Rib Mountain, wound care clinic documentation, and hospital documentation. Based on record review and interviews, Surveyor substantiated the provider did not provide adequate care and services to Resident 1. On 06/11/2026, Resident 1 was admitted to the CBRF. An assessment with an observation date of 06/17/2026 and a completion date of 06/24/2026, by Registered Nurse (RN) C, indicated Resident 1 had an abrasion to the left knee and right elbow. No other skin integrity concerns were identified. Resident 1 was hospitalized with various wounds on 07/23/2026. Resident 1 did not return to the CBRF. Documentation from the hospital indicated Resident 1 died on 08/09/2026. On 08/25/2026 at 2:30 p.m., Surveyor interviewed Administrator A on the phone. Surveyor asked	N 214		

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N 214	Continued From page 3 Administrator A if COO B would be joining on the phone interview. Administrator A told Surveyor, COO B would not be able to join as s/he was in a meeting. Administrator A told Surveyor, s/he would "fill in" COO B on the interview. Administrator A told Surveyor s/he was still in the process of completing training to become a qualified administrator. Surveyor explained that the Department did not receive notification related to the administrator change and Administrator D was still listed as the administrator. Surveyor asked how often COO B comes into the facility. Administrator A said COO B comes to the facility 4 or 5 times a year. Surveyor asked when the last time COO B was in the CBRF. Administrator A said COO B was in the facility last in May 2026, but that s/he was not certain about that and s/he would have to review his/her emails. The provider did not ensure an administrator supervised the daily operations of the CBRF, to ensure residents received proper care and treatment, that their health and safety were protected and promoted, and their rights were respected. Cross References: N0283 83.26(1) Documentation Of Required Employee Training N0382 83.35(1)(b) Sources Used For Assessment Information N0385 83.35(2) Temporary Service Plan N0389 83.35(3)(d) Service Plans Updated Annually Or On Changes N0415 83.37(2)(d) Documentation Of Medication Administration Y3244 50.09(1)(L) Care	N 214		

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N 283	Continued From page 4	N 283			
N 283	83.26(1) Documentation of required employee training The CBRF shall maintain documentation of all employee training under s. DHS 83.21 and task specific training under s. DHS 83.22 and shall include the name of the employee, the name of the instructor, the dates of training, a description of the course content, and the length of the training. This Rule is not met as evidenced by: Based on record review and interview, the provider did not document Administrator A's required task specific training for assessment of residents under Wis Admin Code § DHS 83.22(1). Findings include: Wis. Admin. Code § DHS 83.22(1) reads: "Assessment of residents. All employees responsible for resident assessment shall successfully complete training in the assessment of residents prior to assuming these job duties. Specific training topics shall include: assessment methodology, assessment of changes in condition, sources of assessment information, and documentation of the assessment." On 07/30/2026, the Department received a complaint with allegations the provider did not ensure a resident received the cares and services s/he required and s/he developed multiple wounds. From 08/13/2026 until 08/25/2026, Surveyor reviewed Resident 1's record from the facility. Surveyor reviewed documentation from the	N 283			

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N 283	Continued From page 5 skilled nursing facility s/he resided at prior to admission to the CBRF, wound care clinic, hospital, and home health agency. Surveyor discovered the following: On 04/12/2026, Resident 1 underwent a spinal decompression surgery on his/her cervical spine (neck region), through the front of his/her neck. On 04/14/2026, Resident 1 underwent a spinal decompression surgery on his/her cervical spine through the back of his/her neck. On 04/24/2026, Resident 1 was admitted to a skilled nursing facility with pressure injuries to both buttocks. A pre-admission assessment, dated 05/19/2026, was signed by Administrator A. On 06/11/2026, Resident 1 was admitted to the facility. On 07/23/2026, Resident 1 was hospitalized with pressure injuries to his/her buttocks, genital area, and both heels. Resident 1 did not return to the facility. On 08/25/2026 at approximately 2:30 p.m., Surveyor interviewed Administrator A on the phone. Administrator A told Surveyor s/he was in the process of completing the courses to become a qualified administrator. Surveyor asked Administrator A if s/he received training in conducting assessments on residents. Administrator A told surveyor that Chief Operating Officer (COO) B and Administrator D trained him/her in conducting assessments. Surveyor requested documentation of this training. Administrator A told Surveyor that it was not documented. Administrator A said it was verbal	N 283		

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N 283	Continued From page 6 training and there was no documentation to provide. Administrator A told Surveyor, s/he should not have admitted Resident 1 to the facility as Resident 1 required a higher level of care then they could provide. The provider did not document Administrator A had successfully completed training in the assessment of residents prior to assuming these job duties.	N 283		
N 382	83.35(1)(b) Sources used for assessment information. Information gathering. The CBRF shall base the assessment on the current diagnostic, medical and social history obtained from the person ' s health care providers, case manager and other service providers. Other service providers may include a psychiatrist, psychologist, licensed therapist, counselor, occupational therapist, physical therapist, pharmacist or registered nurse. The administrator or designee shall hold a face-to-face interview with the person and the person ' s legal representative, if any, and family members, as appropriate, to determine what the person views as his or her needs, abilities, interests, and expectations. This Rule is not met as evidenced by: Based on record review and interview, the provider did not gather information from Resident 1's health care providers to thoroughly review Resident 1's medical history and his/her needs, abilities to determine if the community based residential facility (CBRF) could meet his/her needs.	N 382		

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N 382	Continued From page 7 Findings include: On 07/30/2026, the Department received a complaint with allegations the provider did not ensure a resident received the cares and services s/he required and s/he developed multiple wounds. From 08/13/2026 until 08/25/2026, Surveyor reviewed Resident 1's record from the facility. Surveyor reviewed documentation from the skilled nursing facility, s/he resided at prior to admission to the CBRF, wound care clinic, hospital, and home health agency. Surveyor discovered the following: On 04/12/2026, Resident 1 underwent spinal decompression surgery on his/her cervical spine (neck region), through the front of his/her neck. On 04/14/2026, Resident 1 underwent spinal decompression surgery on his/her cervical spine through the back of his/her neck. On 04/24/2026, Resident 1 was admitted to a skilled nursing facility with pressure injuries to both buttocks. A pre-admission assessment, dated 05/19/2026, was signed by Administrator A. On 06/11/2026, Resident 1 was admitted to the facility. On 07/23/2026, Resident 1 was hospitalized with pressure injuries to his buttocks, genital area, and both heels. Resident 1 did not return to the CBRF. Surveyor reviewed faxed documentation from the	N 382		

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N 382	Continued From page 8 skilled nursing facility that resident 1 resided at prior to admitting to the CBRF. The faxed documentation was dated 06/09/2026 and 06/11/2026. On 08/25/2026 at approximately 2:30 p.m., Surveyor interviewed Administrator A on the phone. Surveyor asked what documentation s/he gathered when s/he conducted Resident 1's preadmission assessment. Administrator A said s/he gathered the information from Resident 1's family. Surveyor asked if s/he was aware of the surgery Resident 1 had on his/her spine. Administrator A said when s/he conducted the preadmission assessment, Resident 1 was not wearing a neck collar. S/he said s/he thought Resident 1 could feed him/herself but would need some assistance, but when Resident 1 was admitted, Resident 1 was a total assist with eating. Surveyor asked what their policy was on notifying a physician since Resident 1 did not have a primary care provider when Resident 1 was admitted to the CBRF. Administrator A said they should not have admitted Resident 1 without him/her having a primary care provider. Administrator A said s/he did not review Resident 1's prior health care documentation until after Resident 1 was admitted to the facility. Administrator A told Surveyor Resident 1 should have gone to a skilled nursing facility as they were not equipped to handle Resident 1's needs.	N 382		
N 385	83.35(2) Temporary Service Plan. Temporary service plan. Upon admission, the CBRF shall prepare and implement a written temporary service plan to meet the immediate needs of the resident, including persons admitted for respite care, until the individual service plan	N 385		

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N 385	Continued From page 9 under sub. (3) is developed and implemented. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 1's temporary service plan (TSP) included a plan to meet Resident 1's immediate needs for a urinary catheter, cervical collar, edema sleeve and risk for pressure injury. This is a repeat deficiency. See Statement of Deficiency JBS011, dated 01/29/2024. Findings include: On 07/30/2026, the Department received a complaint with allegations the provider did not ensure a resident received the cares and services s/he required and s/he developed multiple wounds. On 08/13/2026, Surveyor reviewed Resident 1's record. Resident 1 was admitted to the facility on 06/11/2026 from a skilled nursing facility (SNF). The discharge summary from the skilled nursing facility indicated s/he was admitted to the SNF on 04/24/2026 and discharged on 06/11/2026. Resident 1's discharge summary included the following discharge diagnoses and care plan: Impaired activities of daily living (ADLs) Cervical (neck) spinal cord compression with myelopathy - status post anterior (front) cervical discectomy (removal of disc) and fusion on 04/12/2026 and posterior (back) decompression and fusion on 04/14/2026 Osteoarthritis involving multiple joints Osteoporosis without fracture "Assessment: Patient has remained stable, no	N 385		

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N 385	Continued From page 10 reported neck pain/discomfort or trouble swallowing. [S/he] does report intermittent tingling and numbness to bilateral upper extremities. [S/he] has progressed from requiring Hoyer lift to EZ stand for transfers ... [S/he] is wearing cervical collar at the time of discharge visit. Plan: ...Continue cervical collar as ordered by neurosurgery and follow-up as recommended ..." Hyperlipidemia Hypertension Congestive heart failure "Plan: ...Patient wears compressions sleeves to right arm for edema." Urinary retention status post Foley placement "Assessment: Foley catheter was placed while hospitalized due to concerns for neurogenic bladder and treatment for UTI (urinary tract infection) ... Plan: ...Continue in-house Foley catheter cares and monitor clinically ..." Type 2 diabetes mellitus with complication, without long-term current use of insulin "Assessment: Blood sugars are checked daily and have been consistently under 180. Patient denies any symptoms of hypo (low) or hyperglycemia. Plan: Continue diabetic diet and monitor clinically." "Summary of SNF stay: ...[S/he] wears a cervical collar when out of bed, only removing it for eating ...Patient was admitted to facility with bilateral buttocks pressure injuries, which were treated in-house wound care and followed by [name] wound care clinic. Both areas are now healed and showing 100% skin. Current treatment is standard preventative and protective measures including an air mattress, repositioning, monitoring skin condition with ADLs daily,	N 385		

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N 385	Continued From page 11 continuing vitamin C and multivitamins to maintain wound healing." Surveyor could not locate a TSP in Resident 1's record. On 08/13/2026 at approximately 2:00 p.m., Surveyor interviewed Care Coordinator (CC) E. Surveyor asked CC E where Resident 1's TSP was located. CC E told Surveyor it was hanging in the medication room. Surveyor and CC E went to the medication room to retrieve the TSP. Surveyor asked CC E about Resident 1. CC E told Surveyor Resident 1 was "totally different" than how s/he was when they went to do his/her pre-admission assessment. CC E said Resident 1 did not have neck brace on when they did their assessment, s/he said Resident 1 was sitting up by him/herself and could feed self. CC E said when Resident 1 was admitted to the facility, Resident 1 could not sit up by him/herself and was a "total feeder" and "total cares." On 08/13/2026, Surveyor reviewed Resident 1's TSP. The TSP was not dated. The section titled: "Overall Health" included checkbox areas for: diabetes, dementia, osteoporosis, and does not use call light. None of the boxes were checked. There was an area to write in diagnoses. This area was blank. The section titled: "ADL's" included the following: Dressing - Assist of 2 Bathing - Assist of 2 Grooming/Personal hygiene - Assist of 1 Oral care - Assist of 1 Bed Mobility: Assist of 2 Transfer: Assist of 2 Toileting: Assist of 1 - This area had check boxes for: Continent, Incontinent, Bowel, Bladder, Foley,	N 385		

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N 385	Continued From page 12 and Colostomy. None of these areas were marked despite Resident 1 having a Foley urinary catheter. Eating: Assist of 1 - This area had checkboxes for: Diet: General or Other. Neither box was marked. Ambulation: Assist of 2 Assistive Devices: Wheelchair was check marked and "power motor wheelchair" was handwritten. Skin care: This was blank The section titled, "Resident Goals" was blank. Resident 1's TSP did not include his/her cervical collar, edema sleeves, Foley urinary catheter, history of pressure injuries with interventions to prevent pressure injuries from developing. On 06/28/2026, staff documented: "Writer noticed an open sore on resident [sic] right big toe and an open sore on residents [sic] left heel..." On 08/18/2026, Surveyor reviewed Resident 1's record from the medical clinic and hospital. On 06/29/2026, Resident 1 presented to the walk-in clinic with family members with complaints of pressure injuries. The note read, in part: "[Resident 1] is primarily here today for wound checks and referrals for primary care as well as wound care... Wound care provided by nursing staff: Left toe cleansed, thin layer of antibiotic ointment and nonstick dressing applied. Nonstick dressing applied to right heel. Right elbow skin tear cleansed, thin layer of antibiotic ointment and nonstick dressing applied. The patient's incontinence briefs were soiled on exam. The skin was cleansed and a clean brief was applied. I do have a concern for developing cellulitis of the left foot as there [sic] erythema	N 385		

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N 385	Continued From page 13 (redness) and warmth to the left foot, beyond the left toe sore... provided with a prescription for barrier cream to apply to the sore on the right buttock. They are encourage to applied [sic] 3 times daily and additionally if the skin becomes sole [sic] the stool or urine..." The note indicated a referral was placed for wound care and for a primary care physician. On 07/01/2026, Resident 1 was seen at the wound clinic. The note from the wound clinic indicated Resident 1 had developed the following wounds: Stage 2 pressure injury to the left buttock, with possible indications of deep tissue injury Stage 2 pressure injury to the genital area Skin tear to both the left and right elbow Pressure injury to both the left and right heel (not staged) Diabetic ulcer to the left great toe On 08/25/2026 at approximately 2:30 p.m., Surveyor interviewed Administrator A on the phone. Surveyor explained concerns that Resident 1's TSP did not include a plan to meet all of Resident 1's immediate needs. Administrator A told Surveyor that either him/herself or the care coordinator completes the TSP.	N 385		
N 389	83.35(3)(d) Service plans updated annually or on changes Individual service plan review. Annually or when there is a change in a resident 's needs, abilities or physical or mental condition, the individual service plan shall be reviewed and revised based on the assessment under sub. (1). All reviews of the individual service plan shall include input from	N 389		

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N 389	Continued From page 14 the resident or legal representative, case manager, resident care staff, and other service providers as appropriate. The resident or resident ' s legal representative shall sign the individual service plan, acknowledging their involvement in, understanding of and agreement with the individual service plan. This Rule is not met as evidenced by: Based on record review and interview, the provider did not review and revise Resident 1's comprehensive individual service plan (ISP) when s/he had a significant change in condition. Resident 1's (ISP) was not updated when s/he developed multiple wounds. His/her wounds worsened and s/he was hospitalized. Findings include: On 07/30/2026, the Department received a complaint with allegations the provider did not ensure a resident received the cares and services s/he required and s/he developed multiple wounds. On 08/13/2026 at approximately 10:40 a.m., Surveyor interviewed Caregiver F. Surveyor asked about Resident 1. Caregiver F told Surveyor Resident 1 no longer resided at the facility. Surveyor asked Caregiver F about Resident 1 and his/her needs. Caregiver F told Surveyor, Resident 1 was bed bound and that s/he required wound care. Surveyor asked if Resident 1 was admitted to the facility with wounds. Caregiver F said Resident 1 did not have open sores when s/he was admitted, but the wounds started right away after admission.	N 389		

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N 389	Continued From page 15 On 08/13/2026 at approximately 11:00 a.m., Surveyor interviewed Care Coordinator (CC) E about Resident 1. CC E told Surveyor Resident 1 should not have been admitted to the facility. S/he said the previous facility, a skilled nursing facility, told them Resident 1's wounds were healed. CC E said Resident 1's bottom has a "purple coating" and just skin covering the wounds. CC E told Surveyor when Resident 1 went to the hospital his/her wounds needed a wound vac. On 08/13/2026, Surveyor reviewed Resident 1's record. On 06/11/2026, Resident 1 was admitted to the facility. A discharge summary from a skilled nursing facility (SNF) indicated Resident 1 was previously at the SNF from 04/24/2026 until 06/11/2026 following a hospital stay for cervical spine decompression surgeries that took place on 04/12/2026 and 04/14/2026. The discharge summary indicated Resident 1 was admitted to the SNF with bilateral buttocks pressure injuries that were healed in-house. The summary read, in part: "Patient also was admitted to facility with bilateral buttocks pressure injuries, which were treated by in-house wound care and followed by [name] wound care clinic. Both areas are now healed and showing 100% skin. Current treatment is standard preventative and protective measures including an air mattress, repositioning, monitoring skin condition with ADLs (activities of daily living) daily, continuing vitamin C and multivitamins to maintain wound healing." Resident 1's temporary service plan (TSP) did not include Resident 1's history of pressure injuries or risk of pressure injuries.	N 389		

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N 389	Continued From page 16 Resident 1's ISP, with a review and revise date of 07/23/2026, included the following: "Problem Start Date: 06/11/2026 Category: Pressure Ulcer/Injury Resident has a pressure ulcer R/T (related to) deconditioning. Stage: 2 Measurements: 5.0 cm (centimeter) x 2.0cm x 0.2cm Location: Coccyx and right heel. Long Term Goal Target Date: 01/13/2027 Resident's ulcer will heal without complications. Approach Start Date: 06/11/2026 Assess the pressure ulcer for location, stage, size (length, width, and depth), presence/absence of granulation tissue epithelization daily Approach Start Date: 06/11/2026 Avoid friction and shearing forces during transfers or position changes. Approach Start Date: 06/11/2026 Keep bony prominences from direct contact with one another with pillows Every Shift; Approach Start Date: 06/11/2026 Keep clean and dry as possible. Minimize skin exposure to moisture. Approach Start Date: 06/11/2026 Keep linens clean, dry, and wrinkle free. Approach Start Date: 06/11/2026 Observe and report signs of cellulitis (e.g., localized pain, redness, swelling, tenderness, drainage, fever, chills, malaise, tachycardia, hypotension). Approach Start Date: 06/11/2026 Observe and report signs of sepsis (fever, lassitude or malaise, change in mental status, tachycardia, hypotension, anorexia, nausea, vomiting, diarrhea, headache, lymph node tenderness/enlargement). Approach Start Date: 06/11/2026 Provide incontinence care after each incontinent episode."	N 389		

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N 389	Continued From page 17 Resident 1's progress notes included the following: 06/24/2026 - "RN (registered nurse) assessment completed. No concerns at this time." E-signed by Registered Nurse (RN) C. 06/28/2026 - "Writer noticed an open sore on resident [sic] right big toe and an open sore on residents [sic] left heel ..." 07/01/2026 - "Today the resident was seen today by the wound clinic PCP (primary care physician) states resident breathing was a little shallow and states for the residents [sic] to use [his/her] breathing tract [sic] from the hospital and also resident is to have [his/her] foam boots on at all times and heels floating while in bed pillow is to [sic] under [his/her] feet while [s/he] is in [his/her] recliner. Resident started new antibiotic and is to take the probiotic two hours later to stop the loose stools and to continue with the barrier cream three times a day>[sic] Also the resident is to only be in [his/her] wheelchair for two hours at a time and place back in bed for repositioning every two hours." E-signed by CC E. 07/07/2026 - "When assisting resident with toileting during HS (bedtime) cares writer noticed an open sore on residents inner L side of [his/her] buttocks dark blue and purple in color in two different areas ... " 07/08/2026 - "Wound to right side of buttock onto [genitals] noted, Barrier cream applied. Wounds to heels noted, were still in dressings from wound care. Will be reaching out to wound care for direction with dressings and buttocks." E-signed by RN C.	N 389		

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N 389	Continued From page 18 07/09/2026 - "Today [sic] the resident was seen by [name] wound clinic they have sent over new orders to pharmacy and facility resident [sic]. Per PCP the resident is to be only in wheelchair when eating meals after meals the resident is to be transferred back to the bed without any briefs or depends on while in bed resident is only to have a chuck pad under [him/her] [s/he] is not to be in [his/her] recliner staff is to apply calamyz [sic] twice daily on buttock. Resident will be admitted to [name] home health 7/10/26." E-signed by CC E. 07/22/2026 - "New area of redness/breakdown to left hip noted by home health nurse this morning. Reposition every 2 hours to assist with healing of buttocks wounds but also keeping off left hip. Can apply thin layer of barrier cream to area of redness to prevent further breakdown. RN Quarterly Assessment Completed." E-signed by RN C. 07/23/2026 - "Today the resident had appointment with wound clinic today while at wound clinic the nurse called facility and stated [s/he] was going to be admitted to the ED (emergency department) due to the severity of the wound [s/he] has and upcoming show of another states [s/he] will possibly need surgery and that the resident need more assistance than the assisted living can provide at this time ..." E-signed by CC E. On 08/18/2026, Surveyor reviewed documentation from Resident 1's medical provider. On 07/23/2026, Resident 1 had a wound care clinic appointment. Medical Doctor (MD) G documented the following wounds:	N 389		

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N 389	Continued From page 19 "GENITALS: The [genitals] has a small ulceration that measures approximately 1.0 x 1.0 x 0.1 cm (centimeters). It is minimally indurated (hard/firm). It has no inflammation or erythema (redness). BUTTOCK: The right buttock shows an area of ulceration. It measures 5.7 x 6.0 x 0.9 cm. This can not [sic] be staged secondary to the necrosis of the skin covering it. There is induration and mild inflammation. There is no erythema. Of note, this is where the elastic band of [his/her] diaper would overlie. HEELS: The left heel has a small area of eschar (dead tissue) that measures 1.2 x 1.3 cm. There is minimal induration and no inflammation or erythema. The right heel is covered with a thick eschar. It measures 0.4 x 5.5 cm. It is lifting at the edges. There is exposed fat. There is minimal induration. No inflammation or erythema. SACRUM (tailbone area): The sacrum has a new ulcer. It measures 3.3 x 7.4 x 0.2 cm. It is unstageable secondary to obvious deep tissue injury. There is surrounding induration and minor inflammation. There is no erythema. EXTREMITIES: The bilateral upper arms show areas of bruising. There is no real open ulcers or wounds. INTEGUMENT (skin): The patient has quite compromised turgor and integrity to [his/her] skin everywhere. It is consistent with age." On 08/19/2026, Surveyor emailed Administrator A and Chief Operating Officer (COO) B. Surveyor requested additional documentation. The email read, in part: "Were there any other ISPs or Care Plans between [Resident 1's] temporary service plan and the ISP, dated 07/23/26? Please email with dates of revision if there were." On 08/19/2026, Administrator A emailed Surveyor which included a copy of Resident 1's ISP with	N 389		

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N 389	Continued From page 20 dates of revision. The approaches for Resident 1's pressure injuries were not added to Resident 1's ISP until 07/23/2026, when Resident 1 was hospitalized. Resident 1's ISP did not include all of his/her wounds or interventions that were in Resident 1's progress notes to reposition, to only be up for meals, to float heels, wear foam boots, to leave the incontinent brief off while in bed, to apply calazime cream. On 08/25/2026 at approximately 2:30 p.m., Surveyor interviewed Administrator A on the phone. Surveyor explained that when Resident 1's wounds were first identified, Resident 1's ISP was not reviewed and revised to implement interventions to promote healing. Administrator A told Surveyor that s/he understood now that the residents' ISPs needed to be reviewed and revised immediately when they had a change in condition.	N 389		
N 415	83.37(2)(d) Documentation of medication administration. Documentation of medication administration. As required under s. DHS 83.42(1)(o), at the time of medication administration, the person administering the medication or treatment shall document in the resident record the name, dosage, date and time of medication taken or treatments performed and initial the medication administration record. Any side effects observed by the employee or symptoms reported by the resident shall be documented. The need for any PRN medication and the resident 's response shall be documented.	N 415		

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N 415	Continued From page 21 This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure staff documented in Resident 1's record the name, dosage, date and time of medication taken or treatments performed and initial the medication administration record. Resident 1 was prescribed an antibiotic, for a urinary tract infection, prior to his/her admission on 06/11/2026. There was no documentation to indicate Resident 1 received the antibiotic when s/he was admitted. Findings include: On 07/30/2026, the Department received a complaint with multiple allegations including concerns related to medication administration. On 08/13/2026, Surveyor reviewed Resident 1's record. Resident 1 was admitted to the facility on 06/11/2026. Resident 1's record contained a fax from Resident 1's previous skilled nursing facility. The fax had a date stamp of 06/11/2026. The faxed documentation was a discharge summary that included Resident 1's discharge visit by an advance practice nurse practitioner (APNP) on 06/05/2026. There was an addendum, dated 06/09/2026, which read, in part: "Patient developed the onset of hallucinations and confusion 6/7/2026... Urine culture positive for Klebsiella Oxytoca UTI. Patient started on ciprofloxacin 500 (milligrams) p.o. (by mouth) twice daily x 5 days..." The discharge summary	N 415		

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N 415	Continued From page 22 indicated Resident 1 had urinary retention with a Foley catheter. Under the section for discharge medications, a note read: "Patient's medications have not been prescribed to their pharmacy of choice assisted living facility indicates they will order medications." On 08/17/2026, Surveyor emailed Administrator A and Chief Operating Officer (COO) B. Surveyor requested additional documentation to review for the survey. The email read, in part: "Please send the medication administration record for the ciprofloxacin, [Resident 1] was prescribed on 06/09/2026." On 08/17/2026, Administrator A emailed Surveyor Resident 1's medication administration record (MAR) for ciprofloxacin that was prescribed 06/20/2026- 06/30/2026. Surveyor emailed Administrator A and requested Resident 1's MAR for the prescribed ciprofloxacin for Resident 1 was prescribed when s/he was first admitted. Administrator A replied via email. The email indicated they did not have an order to administer the ciprofloxacin until 06/20/2026. On 08/25/2026 at approximately 2:30 p.m., Surveyor interviewed Administrator A. Surveyor asked why Resident 1 did not receive the ciprofloxacin antibiotic upon admission (06/11/2026) when s/he was prescribed the ciprofloxacin on 06/09/2026, prior to his/her admission. Administrator A told Surveyor s/he remembered staff administering the ciprofloxacin to Resident 1 when s/he was admitted. Administrator A told Surveyor Resident 1's family brought in the bottle of ciprofloxacin for staff to administer. Surveyor asked if there was documentation to indicate the ciprofloxacin was administered to Resident 1 upon admission.	N 415		

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Y3244	Continued From page 24 s/he required and s/he developed multiple wounds. From 08/13/2026 until 08/25/2026, Surveyor reviewed Resident 1's record, including records from the following outside agencies: walk in clinic, wound care clinic, emergency room, hospital, and home health. Resident 1 was admitted to the facility on 06/11/2026. S/he did not have an activated healthcare power of attorney or guardian and was able to make his/her own medical decisions. Resident 1 had multiple diagnoses that included: impaired activities of daily living, cervical cord compression with myelopathy (pain, numbness, and difficulty moving parts of the body), osteoarthritis, hypertension, congestive heart failure, and type 2 diabetes. Prior to his/her admission, Resident 1 was at a skilled nursing facility. Documentation from the skilled nursing facility indicated Resident 1 had a spinal surgery through the front and back of his/her neck and was admitted to the skilled nursing facility with pressure injuries to both buttocks. The discharge summary indicated the pressure injuries were treated and healed at time of Resident 1's discharge from the skilled nursing facility. An assessment, with an observation date of 06/17/2026 and a completion date of 06/24/2026, by Registered Nurse (RN) C, included a section for skin condition. This section included the following areas: "Does the resident have any of the following risk factors for skin breakdown" Check marked were: decreased activity level, immobility, incontinence, poor nutrition, and poor skin turgor. RN C did not check Predisposing diseases that included diabetes and arthritis. RN	Y3244		

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Y3244	Continued From page 25 C did not check mark history of pressure sores. "Character of Skin Color and Temperature" Check marked were: per usual and warm. "Skin Integrity" Check marked were: Other open areas, scars, and moles. "Describe any additional body marks or sores as necessary" "Abrasion" was documented. "Location of Impaired Skin Integrity" "left knee, right elbow" was documented. "Foot Problems and Care" Check marked was none of the above. On 06/24/2026, Registered Nurse (RN) C documented: "RN admission assessment completed. No concerns at this time." On 06/28/2026, Caregiver F documented: "Writer noticed an open sore on resident [sic] right big toe and an open sore on residents [sic] left heel. Writer applied cream and bandaged the sore. [Family] was at facility at the time and is aware of the sores. Family will be taking resident to walk in first thing in the morning." There was no documentation to indicate the provider notified Resident 1's physician. Documentation from the walk in clinic indicated Resident 1 presented to the walk in clinic on 06/29/2026. The documentation, read, in part: "The patient is a [age] [fe/male] who presents with [family names] to the [name] Walk-in with complaints of pressure ulcers. Has poor mobility and is in a wheelchair. In April of this year patient underwent a cervical spine procedure and hospitalization ... Was then transferred to a rehab facility for recovery and now is living at Wellington Place Assisted Living. Both of [Resident 1's] children are concerned about pressure ulcers.	Y3244		

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Y3244	Continued From page 26 Would like these checked on today. [S/he] is now going to be living in this area and they are interested in establishing care with a primary care provider and also looking for wound care referral ... Chief complaint and physical exam findings were discussed with the patient and children. [Resident 1] is primarily here today for wound checks and referrals for primary care as well as wound care ... Images of wounds above. Wound care provided by nursing staff: Left toe cleansed, thin layer of antibiotic ointment and a nonstick dressing applied. Nonstick dressing applied to right heel. Right elbow skin tear cleansed, thin layer of antibiotic ointment and nonstick dressing applied ... Is provided with a prescription for barrier cream to apply to the sore on the right buttock ... Nursing staff is attempting to get patient scheduled with a primary care provider this week for close follow up." On 07/01/2026, Resident 1 was seen at the wound care clinic. Medical Doctor (MD) G documented, in part: "CHIEF COMPLAINT Pressure ulcers, buttocks and heel. HISTORY OF THE PRESENT ILLNESS The patient is a very debilitated [age] [fe/male]. [S/he] had for some time been suffering from progressive weakness. [S/he] had for some time been going to physical therapy. Beginning in late March, [s/he] started to develop progressive weakness. Within that 2 week period [his/her] weakness became so severe [she] could not lift [his/her] arms up to feed [him/herself] ... On April 10, 2026, [s/he] was taken to the [medical clinic] in [city]. [S/he] was found to have urinary retention, an inability to feed [him/herself], dysphagia (difficulty swallowing) and tremendous generalized weakness. [His/her] workup revealed significant degenerative changes of C2 and C3	Y3244		

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Y3244	Continued From page 27 (2nd and 3rd bones in the cervical spine). There was significant spinal cord compression. [S/he] had a cervical discectomy (removal of disc), osteophyctectomy (removal of bony growth) and decompression with instrumentation on April 12. [S/he] had a second cervical decompression on April 14. [S/he] was seen by the wound care team on April 13. [S/he] had pressure injuries to [his/her] bilateral buttocks and multiple skin tears. Review of the notes on May 18, 2026, by the wound care team indicated that the skin tears and the bilateral buttock pressure ulcers had healed. They did note that [his/her] current medical condition left [him/her] at risk for continued development of pressure injuries. They advocated offloading through positioning and cushioning. [S/he] was discharged from the hospital to a skilled nursing facility. [S/he] was there for quite some time receiving therapy. [S/he] eventually left there and is now in an assisted living facility, Wellington, ALF (assisted living facility), in Rib Mountain. The patient presented to [name] Walk-in Clinic on June 29, 2026. This was shortly after [s/he] was transferred to the Wellington facility. This was to enable [him/her] to be closer to [his/her] children. At this time [s/he] was found to have wounds on [his/her] buttocks and feet. These were addressed. [S/he] was placed on Keflex. [S/he] was referred to this clinic. [S/he] presents today complaining of no pain ... [S/he] spends all of [his/her] time in bed or a chair. The family has an [sic] acquired a wheelchair and a cushion on their own without medical advice ... ASSESSMENT The patient is a quite debilitated [age] [fe/male] who presents with multiple medical problems in addition to skin tears and pressure ulcers. DIAGNOSES 1. Pressure injury/ulcer, stage II, partial thickness,	Y3244		

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Y3244	Continued From page 28 breakdown of skin, left buttock, with possible indications of deep tissue injury. 2. Stage II pressure injury/ulcer, partial thickness, breakdown of skin, [genitals]. 3. Skin tear, partial thickness, left elbow. 4. Skin tear, partial thickness, right elbow. 5. Pressure injury/ulcer, are [sic] partial thickness, breakdown of skin, right heel. 6. Pressure injury/ulcer, are [sic] partial thickness, breakdown of skin, left heel. 7. Diabetic ulcer, left great toe, partial thickness, breakdown of skin ... MEDICAL DECISION MANAGEMENT ...This leads us to the first major concern. [S/he] needs a higher standard of care. Assisted living facilities even with the use of Visiting Home Nursing is not adequate for this patient. I addressed this with [his/her] [family]. We will reach out to Social Work and have them contact the family. The patient is bed and chair bound. [S/he] would benefit greatly from cushioning to offload the areas. We would like to have [him/her] in an alternating air mattress. We would also like to see [him/her] get an appropriate wheelchair with a Roho cushion. This is best handled through a primary care provider. [S/he] has [sic] trying to establish care locally ... We believe [s/he] has a protein malnutrition. We believe this began with the inability to eat properly. We believe it is improving. But we would encourage increased protein intake ... Regarding skin tears to the elbows and left knee we will clean these with a gentle soap and water. We then have [him/her] apply Xeroform (non-adhesive petroleum gauze) and bordered foams. We would have them change this 3 times weekly. For the great toe, we would again wash with soap and water. We would apply Acticoat 7 (silver antimicrobial dressing). We would apply an ABD (absorbent dressing). We would also do this to the heel ulcers except we	Y3244		

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Y3244	Continued From page 29 would form the ABD into a heel cup. We would secure it with rolled gauze. The buttock and [genitals] are more concerning. We do not believe that it will be possible to secure dressings there. We are concerned about the edges of the buttock ulcer. Interestingly the right buttock ulcer is in the crease to the left thigh. It looks as though it could be consistent with injury from the elastic of [his/her] diaper. We would suggest the diaper should be kept very loose. Perhaps they can use something else. The elastic would not go over the [genitals], but again would rub in that area on the outside. We will just apply Calazime. The heel blisters need to be debrided (removal of dead/damaged tissue) ... We believe they are from rubbing on the sheets. This is based on the position of the ulcers. Once we debride them we will put them in Acticoat 7 and heel cups as stated. We, however, will add sage boots to protect and offload these areas." On 07/01/2026, the wound care clinic RN, documented the following note: "Wound Clinic Plan of Care: Expected Wound Outcomes: Healing Wound Progress/Lack of Progress: Initial visit to wound clinic for multiple wounds Return Appointment/Follow-Up: One week with [MD G] Local Anesthetic Use for Wound Care: Not applicable Does patient have home health? Referral placed to [name] Home Health then cancelled (Spoke with [name] at [name] Home Health). Referral sent via fax to Wellington place per their staff request. See comments below Was DME (durable medica equipment) ordered? No, Home health to obtain Comments: Pre and Post debridement measurements of left and right heel not recorded.	Y3244		

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Y3244	Continued From page 30 Provider updated. Patient has bilateral heelmedix (brand of foam boots) boots at home. Call place to [Administrator A] at Wellington Place. [S/he] shares that patient already has home health services through [home health agency]. [S/he] asks that orders be sent directly to them, fax [number] and they will give to [home health agency] RN [RN C] who is in their building today." On 07/01/2026, Care Coordinator (CC) E documented: "Today the resident was seen today by the wound clinic PCP (primary care physician) states resident breathing was a little shallow and states for the residents [sic] to use [his/her] breathing tract [sic] from the hospital and also resident is to have [his/her] foam boots on at all times and heels floating while in bed pillow is to [sic] under [his/her] feet while [s/he] is in [his/her] recliner. Resident started [sic] new antibiotic and is to take the probiotic two hours later to stop the loose stools and to continue with the barrier cream three times a day> [sic] Also the resident is to only be in [his/her] wheelchair for two hours at a time and place back in bed for repositioning every two hours." Resident 1's individual service plan (ISP), last reviewed and revised 07/23/2026, did not include these interventions. On 07/07/2026 at 6:42 a.m., staff documented: "When assisting resident with toileting during HS (bedtime) cares writer noticed an open sore on residents [sic] inner L (left side) of [his/her] buttocks dark blue and purple in color in two different areas ..." There was no documentation to indicate the provider notified Resident 1's physician.	Y3244		

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Y3244	Continued From page 31 On 07/08/2026 at 7:14 a.m., staff documented: "Resident was up visiting with [his/her] [family] in [his/her] room upon this staff arrival after supper. Writer was informed by family that resident has a [sic] appointment coming up within the week to the wound clinic and family will be updating the doctors with concerns of residents [sic] bottom and open pressure wounds that appear to be old but spreading ..." There was no documentation to indicate the provider notified Resident 1's physician. On 07/08/2026, RN C documented: "Wound to right side of buttock onto [genitals] noted, Barrier cream applied. Wounds to heels noted, were still in dressings from wound care. Will be reaching out to wound care for direction with dressings and buttocks." On 07/09/2026, Resident 1 was seen at the wound care clinic. MD G documented, in part: "[S/he] has worsening of the right buttock ulcer. It shows signs of deep tissue injury. Unless there is a dramatic change, it will need surgical debridement. It will continue to grow larger." On 07/09/2026, Resident 1 was seen by MD G in the wound clinic. The wound care clinic RN, documented the following note: "Expected Wound Outcomes: Healing Wound Progressing Toward Wound Outcome: No Progress/Lack of Progress: Buttock wound increased in size/worse condition, all other wounds dry due to dressing changes not being done since last Wound Clinic appointment (see below) Return Appointment/Follow-Up: Monday/Thursday with Wound Nurse ([Family] will cancel if Home Health is started), [MD G] in	Y3244		

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Y3244	Continued From page 32 7-10 days. Local Anesthetic Use for Wound Care: Not applicable Does patient have Home Health> Yes, [home health agency] did not start. Called Wellington Place Rib Mountain to check when dressings were last changed. They state they have not been changed since last Wound Clinic appointment. [S/he] states that they did not receive orders from Wound Clinic (despite successful fax confirmation in EPIC (electronic health record)). Writer requests that in the future, if they do not get orders following a wound clinic appointment to call the clinic. Writer called (home health agency) and they state that they did not start services due to patient not having a PCP currently. Patient has an appointment to establish with a PCP on 7/29/26. They state they updated Wellington on this. Writer updated [MD G] and [s/he] will sign the face to face so services can be started. Was DME ordered? No, home health will provide Comment: Suspected deep tissue injury to left 1st metatarsal head and proximal to both heel wounds. Monitor at next appointment." The wound care clinic documentation, from 07/09/2026, indicated the following wounds: Unstageable, deep tissue injury to left buttock. Stage 2 pressure injury to genitals. Multiple skin tears, both arms. Stage 2 pressure injury to left heel. Stage 3 pressure injury to right heel. Documentation from the home health agency included a fax, dated 07/01/2026, from CC E. The fax included wound care instructions for Resident 1 from the wound care clinic, dated 07/01/2026. The orders read: "ACTIVITY	Y3244		

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Y3244	Continued From page 33 You may shower with waterproof cover over dressing. Keep pressure off of all wound areas. Use pressure reducing boots to bilateral feet when up in wheelchair. Unsure [sic] heels are not touching the mattress when in bed. WOUND CARE INSTRUCTIONS "Cleanse all wounds with normal saline and pat dry. Then follow instructions below Left and Right elbows: Apply single layer of Xeroform and cover with bordered foam. Change 3x weekly Left great toe: Apply Acticoat 7, ABD heel cup, stockingette or 2 inch rolled gauze. Change 3x weekly Right buttock and posterior [genitals] : Apply Calazime Cream in nickel thick layer. Do not rub / scrub off. If cream becomes soiled, use gentle cleanser to remove. Pat and then reapply cream. Left and Right heels: Apply fenestrated Acticoat 7, ABD heel cup, rolled gauze. Secure with tape and change 3x weekly Left knee: Apply xeroform and bordered foam. Change 3x weekly A referral to [home health agency] was placed today, 3x weekly wound cares requested. May see patient 2x weekly on weeks [s/he] has wound clinic appointments. Wellington Place staff: Please apply Calazime Cream twice daily to maintain nickel thick layer. Do not rub / scrub off if cream becomes soiled. Use gentle cleanser to remove any stool, then pay [sic] dry and re-apply cream. Follow up in wound clinic with [MD G] in one week. Call with any questions or concerns prior to next appointment." The documentation from the home health agency indicated Resident 1 did not enroll to receive	Y3244		

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Y3244	Continued From page 34 home health services until 07/10/2026. RN C documented an assessment, dated 07/10/2026. RN C is the provider's nurse and the home health agency nurse. On 07/13/2026, Resident 1 was seen by the home health nurse, RN H. RN H documented, in part: "Patient seen for routine SN (skilled nursing) visit. Wound care completed to pressure injuries as ordered. Patient was noted to have a new stage 2 pressure injury to left buttock. See flow sheets for all wound specifics. Call placed to wound clinic and received orders to continue same treatment as other buttock wound." On 07/15/2026, Resident 1 was seen by the home health nurse, RN C. RN C documented, in part: "Patient seen today for routine skilled nursing visit. All wounds were cleaned and redressed per order." On 07/17/2026, Resident 1 was seen by the home health nurse, RN H. RN H documented, in part: "Patient seen for routine SN visit. Wound care completed to pressure injuries as ordered. Patient was noted to have a new area of MASB (moisture associated skin breakdown) to left buttock. See flow sheets for all wound specifics. Call placed to wound clinic and received orders to continue same treatment as other buttock wounds." On 07/20/2026, Resident 1 was seen by home health nurse, RN C. RN C documented, in part: "Patient seen for routine SN visit. Wound care completed to pressure injuries as ordered. Left knee abrasion is resolved. Buttocks wounds now have moderate smelling odor. Other wounds remain stable and show no s/s (signs and symptoms) of infection. Next wound care appt	Y3244		

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Y3244	Continued From page 35 scheduled for 7/23/26." RN C did not document if Resident 1's physician was notified of the moderate smelling odor, which could indicate an infection. On 07/22/2026, Resident 1 was seen by home health nurse, RN H. RN H documented, in part: "Patient seen for routine SN visit. Wound care completed to pressure injuries as ordered. Stage 2 pressure injury to left buttock has progressed to a stage 3. See flow sheets for all wound specifics. Call placed to wound clinic and updated on wound status." On 07/23/2026, Resident 1 was seen at the wound care clinic. MD G documented, in part: "When we saw [him/her] on July 1, [s/he] had multiple areas of minor injuries. However, [s/he] obviously exhibited the need for a higher level of care. We attempted to acquire this. In the meantime [s/he] has dramatically decompensated. [His/her] stage II pressure ulcer of the buttocks is now on [sic] unstageable and needs to be surgically debrided. In light of [his/her] clinical situation and the need for surgical intervention we will have [him/her] admitted ... DIAGNOSES 1. Pressure injury/ulcer, unstageable secondary to deep tissue injury, left buttocks. 2. Stage II pressure injury/ulcer, partial thickness, breakdown of skin, [genitals], improving. 3. Multiple skin tears, bilateral arms, resolving. 4. Pressure injury/ulcer, stage II, partial thickness, breakdown of skin, left heel, resolving. 5. Pressure injury/ulcer, stage III, exposed fat, right heel ..." Documentation from the hospital indicated	Y3244		

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Y3244	Continued From page 36 Resident 1 was sent to the emergency department from the wound care clinic. S/he was then admitted to the hospital. A flowsheet for Resident 1's wounds indicated wounds were removed from flowsheet on 08/09/2026 "because the patient is deceased." On 08/17/2026, Surveyor emailed Administrator A and Chief Operating Officer (COO) B and requested additional information. The email read, in part: "Please send any wound documentation and treatments that were provided to [Resident 1] while [s/he] resided at Wellington." On 08/18/2026, Administrator A replied via email, which read, in part: "Further investigation related to the residents [sic] wounds have found the following: A referral was made to the local home health agency on 7/1 for wound care. Please see attached. Upon review - the orders for Wellington staff were to apply Calazime cream - all other orders related to wound care were directed at the wound clinic and home health staff. Albeit these orders should have been carried to the EMAR (electronic medication administration record) with directions for home health to change. When speaking to the team, they did not believe they needed to include wound care orders as it was not something they were going to be responsible for. In speaking to [home health agency] administrator [sic] the following occurred; The referral was sent to [name] home health, but the wound care clinic was told [s/he] was already on our services which was not accurate. Once we received the referral, I [sic] we spoke with the	Y3244		

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Y3244	Continued From page 37 wound clinic and [his/her] PCP because we didn't have a following MD nor a face to face and the wound clinic was refusing to sign orders or anything related. [His/her] main provider was from [city], and [s/he] refused to sign any orders as [s/he] wasn't seen by [him/her] for a while. We talked wound care into signing the face to face and following wound care orders only. Once we got that confirmation we admitted [him/her] onto services on the 10th. We assumed the facility shared the orders with the RN who visits the facility, but from the notes it looks like [s/he] followed up on the 8th as [s/he] didn't know we received them. We got the face to face and the agreeance from the wound clinic on the 9th and admitted the next day knowing the urgency. The resident was seen at the wound clinic between the 1st and the 10th. Other than the progress notes, I don't have anything to add on the wound care other than we did monitor to ensure the dressings were on and did follow the offloading instructions. Staff also followed the orders related to the Calazime cream." On 08/19/2026, Surveyor emailed Administrator A and COO B and asked for documentation to indicate Resident 1's Calazime cream was ordered and administered. Surveyor did not receive documentation that the Calazime cream was ordered and administered as ordered. On 08/25/2026 at approximately 2:30 p.m., Surveyor interviewed Administrator A. Administrator A told Surveyor s/he was aware Resident 1 had surgery and had wounds prior to his/her admission, but s/he said s/he thought Resident 1 "was out of the dark" as it related to	Y3244		

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Y3244	Continued From page 38 his/her wounds. Administrator A said s/he admitted him/her to the facility as family wanted Resident 1 to come to the facility. S/he said the CBRF (community-based residential facility) was not a good fit. S/he said they were not able to meet his/her clinical needs. Surveyor asked if s/he located any documentation to indicate staff administered the Calazime cream as ordered. Administrator A told Surveyor there was no documentation of the Calazime. Surveyor explained concerns that Resident 1 did not receive wound care between his/her wound care visits from 07/01/2026 until 07/09/2026. Administrator A said RN C was in the building during that time. The provider failed to provide Resident 1 with adequate and appropriate care to prevent wounds and to promote healing once wounds were discovered. Resident 1's wounds progressed. Resident 1 was hospitalized and did not return to the facility. Cross Reference: N0389 83.35(3)(d) Service Plans Updated Annually Or On Changes	Y3244		